

Diabetes & oral surgery

SPECIAL INSTRUCTIONS FOR DIABETIC PATIENTS · SEDATION & RECOVERY

Pairs with your
procedure-specific
recovery guide

Having diabetes doesn't prevent you from having oral surgery — but it does change a few things about how we prepare, manage sedation, and care for you afterward. **Tell us about all your diabetes medications — including any GLP-1 drugs like Ozempic, Wegovy, or Mounjaro — well before your procedure date.** If you have any questions about this guide, call our office before making changes to your medications.

Which parts apply to you: the *Before your surgery* and *Day of surgery* sections apply only if you are being **sedated** (one exception: the SGLT-2 box, which depends on the size of your surgery, not on sedation). *After your surgery* applies to everyone with diabetes having oral surgery, sedated or not.

⚠ If you take a GLP-1 medication **CRITICAL FOR SEDATION**

GLP-1 drugs slow down how fast your stomach empties food. Even after a normal fasting period, there can still be food in your stomach — which creates a real risk of aspiration (food entering your lungs) during sedation.

Common GLP-1 medications: Ozempic, Wegovy, Rybelsus (semaglutide) · Mounjaro, Zepbound (tirzepatide) · Trulicity (dulaglutide) · Victoza, Saxenda (liraglutide)

If you are receiving IV sedation: Current national guidance (2024 multi-society statement) says most patients **continue** their GLP-1 — and our office follows that: **keep taking your medication as prescribed; do not skip a dose for surgery.** Because office sedation does not use a protected airway, **our fasting policy is more conservative than national guidance:** follow a **clear-liquid diet for the 24 hours before your fasting time.** If your dose was **recently increased** or you are having **any stomach symptoms**, tell us when you schedule — we may move your appointment for your safety. Call our office *and* your prescribing doctor as soon as you schedule surgery to plan this together.

Day of surgery: if you have nausea, vomiting, bloating, or abdominal pain, tell us before we start — we may need to reschedule for your safety.

If you are not having sedation (local anesthesia only): you typically do not need to stop your GLP-1. Confirm with us either way.

Before your surgery — if you are being sedated

MEDICATION	WHAT TO DO
Metformin	Usually hold the morning of surgery if you're having sedation. Restart once you're eating normally. Confirm with us.
Other oral diabetes pills	(Sulfonylureas like glipizide, glyburide.) Usually hold the morning of surgery because you'll be fasting — taking them without eating can cause low blood sugar. (SGLT-2 pills are different — see the box below.)
Long-acting insulin	(Lantus, Basaglar, Tresiba, Levemir, Toujeo.) Usually take about half your normal dose the night before or morning of surgery. Confirm the exact amount with your prescriber.
Mealtime insulin	(Humalog, Novolog, Apidra, Admelog.) Do not take the morning of surgery since you're not eating. Bring it with you in case we need it afterward.
Insulin pumps	Usually kept on your basal rate . Skip the breakfast bolus. Bring extra supplies and your glucose meter to your appointment.

i SGLT-2 pills (Jardiance, Farxiga, Invokana, Steglatro) — sedated or not. These can cause a dangerous acid buildup (ketoacidosis), even with normal glucose, when you're eating much less than usual. If you're having **several teeth removed**, or surgery that will change how you eat for a while, **stop them 3 days before** (Steglatro: 4 days). For a **single tooth or an implant**, you can usually keep taking them. Confirm timing with your prescriber.

 **Bring to your appointment:** glucose meter, test strips, a fast-acting sugar source (glucose tabs or juice), a snack for after, and a list of your current doses. Arrive with your glucose in a safe range — we'll check it again before starting.

The day of surgery — if you are being sedated

DO

- ✓ Follow your NPO (no food/water) instructions exactly
- ✓ Take blood pressure and heart medications with a tiny sip of water
- ✓ Check your glucose before leaving for your appointment
- ✓ Bring your glucose meter and fast-acting sugar
- ✓ Let us know if your glucose is below 80 or above 250 on arrival

DON'T

- ✗ Take mealtime (short-acting) insulin while fasting
- ✗ Skip your long-acting insulin entirely — take the adjusted dose
- ✗ Eat or drink inside your NPO window

After your surgery

Diabetes affects how wounds heal. A few things that matter more after surgery than day to day:

- **Aim for good glucose control** (generally 80–180) during the first 2 weeks. High glucose slows healing and raises infection risk.
- **Eating will be different for a few days** — soft foods, smaller portions. You may need to adjust insulin doses. Check your glucose more often than usual (every 3–4 hours while awake for the first few days).
- **If you can't eat normally, don't take your usual mealtime insulin.** Match your dose to what you're actually eating. Call your diabetes provider if you're unsure.
- **Hydration matters.** Dehydration raises blood sugar and slows healing. Sugar-free fluids are fine.
- **Infection risk is higher with diabetes.** Follow the Call Us warnings in your procedure handout carefully — especially fever, returning pain, bad taste, or pus. Call at the first sign rather than waiting.

i About pain medications and glucose: Ibuprofen and acetaminophen don't affect blood sugar meaningfully. Steroid dose packs (if prescribed) will raise your glucose for 5–7 days — plan to check more often and adjust insulin as your diabetes provider directs. Antibiotics can cause GI upset that affects eating, which in turn affects glucose.

Call our office right away if...

GLUCOSE PROBLEMS

- Glucose over 250 that won't come down
- Glucose under 70 that you can't correct
- Ketones in your urine (type 1)
- Vomiting or unable to keep fluids down

HEALING CONCERNS

- Any standard warning sign on your procedure handout
- Signs of infection at the surgical site
- Fever over 101°F
- Pain or swelling worsening after day 4

 **Don't hesitate to call.** Diabetic patients sometimes get infections a day or two later than others and with fewer early signs. We'd rather see you for a quick check-in than treat a larger problem later.

Acknowledgment & signature

I acknowledge that I have received and understand these instructions, and all my questions have been answered. I agree to follow them and to call the office with any questions or concerns.

PATIENT SIGNATURE

DATE

PATIENT PRINTED NAME

DATE OF BIRTH

PARENT / LEGAL GUARDIAN / CAREGIVER SIGNATURE (IF SIGNING FOR THE PATIENT)

DATE

PRINTED NAME

RELATIONSHIP TO PATIENT